ICDL - Intraoperative Checklist and Device Log

Case identification, purpose and release to start

ICDL · v0.15

Study / protocol	Site / code	Participant code	ICDL ID
Procedure date	**Procedure / side**	**Operating room**	**Primary surgeon (code)**
Device / system	**CIP and surgical technique (version)**	**Accountability record location**	**Controlled location**

A Purpose and use

The ICDL is filled in during one participant's procedure, not afterwards, because what happens in the operating room cannot be rebuilt reliably later. It records the release to start (B), the critical steps (C), the times and the case data the surgeon's learning curve record needs (D), every component used with its identifiers (E), changes to the plan (F), complications and device problems (G), the review right after surgery (H) and whether the case can be closed (I). Open actions go to J.

It does not replace the operative note, the device accountability record, safety reporting or the formal deviation record; it captures what is first known here and points to them.

Unit of analysis	Decision supported	Inputs	Outputs
One participant and one procedure.	Whether the case can be closed: critical steps, device identity, changes and immediate outcome documented or sent on.	CIP and surgical plan, surgeon release (SCR, TRE), device instructions, accountability materials.	Case entry for the SLCT; components for accountability; events to the SCML and SART; images to the IQL; deviations and CAPA.

B Release to start

All items before the first incision. A No stops the start until it is resolved or a hold is recorded.

Control	Status	Evidence or reference	Exception · hold · route
Surgeon released for this procedure (SCR) and on the delegation log	☐ Yes ☐ No ☐ N/A		
Participant, procedure and side match the schedule and source records	☐ Yes ☐ No ☐ N/A		
Consent, eligibility (PSEAR) and pre-operative checks available	☐ Yes ☐ No ☐ N/A		
Device system, components and instruments available and compatible	☐ Yes ☐ No ☐ N/A		
Packaging, expiry, storage and sterile release acceptable	☐ Yes ☐ No ☐ N/A		
Planning images, measurements and case materials available	☐ Yes ☐ No ☐ N/A		

Release decision	Time	Responsible (name / role) · hold record
☐ Released ☐ Released with observation ☐ Hold		

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Critical steps, times and case data

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C Critical steps

Use the critical steps of the CIP, surgical technique or site checklist. Record any step left out, adapted, repeated or out of order. N/A needs the reason; Partial, Not performed, Not acceptable or Not assessable needs a row in F or J.

#	Critical step	Done	Evidence or reference	Result	Initials · note
1		☐ Complete ☐ Partial ☐ Not performed ☐ N/A		☐ Acceptable ☐ Observation ☐ Not acceptable ☐ Not assessable	
2		☐ Complete ☐ Partial ☐ Not performed ☐ N/A		☐ Acceptable ☐ Observation ☐ Not acceptable ☐ Not assessable	
3		☐ Complete ☐ Partial ☐ Not performed ☐ N/A		☐ Acceptable ☐ Observation ☐ Not acceptable ☐ Not assessable	
4		☐ Complete ☐ Partial ☐ Not performed ☐ N/A		☐ Acceptable ☐ Observation ☐ Not acceptable ☐ Not assessable	
5		☐ Complete ☐ Partial ☐ Not performed ☐ N/A		☐ Acceptable ☐ Observation ☐ Not acceptable ☐ Not assessable	

D Times and case data for the learning curve record

Duration is taken from first incision to closure. Complexity uses the levels defined for the SLCT.

Time point	Time	Condition or status	Initials
First incision			
Device opened / first component used			
Final implant and configuration confirmed			
Closure / end of procedure			

Case data for the SLCT	Entry
Complexity level (source)	
Senior support or proctoring	☐ None ☐ Planned ☐ Unplanned
SLCT entry	

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Components and traceability

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E Components used and traceability

Every component used, including trials, replacements, explanted or discarded ones. Copy identifiers from the label at the time of use; never fill a missing identifier later. Reconcile with the operative note and the accountability record before closure; if it does not reconcile, the case is not closed as complete (F).

#	Component · role	Manufacturer · model	Catalogue no.	UDI-DI	Lot or serial	Size · configuration	Disposition	Label on sheet?
1							☐ Implanted ☐ Trial ☐ Returned ☐ Discarded ☐ Explanted	☐ Yes ☐ No
2							☐ Implanted ☐ Trial ☐ Returned ☐ Discarded ☐ Explanted	☐ Yes ☐ No
3							☐ Implanted ☐ Trial ☐ Returned ☐ Discarded ☐ Explanted	☐ Yes ☐ No
4							☐ Implanted ☐ Trial ☐ Returned ☐ Discarded ☐ Explanted	☐ Yes ☐ No
5							☐ Implanted ☐ Trial ☐ Returned ☐ Discarded ☐ Explanted	☐ Yes ☐ No
6							☐ Implanted ☐ Trial ☐ Returned ☐ Discarded ☐ Explanted	☐ Yes ☐ No
7							☐ Implanted ☐ Trial ☐ Returned ☐ Discarded ☐ Explanted	☐ Yes ☐ No

Final implanted configuration	
Reconciled with (record and reference)	

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Changes, complications and device performance

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F Changes to the plan, substitutions and exceptions

Whenever a planned step, component, condition or sequence is changed, left out, repeated or uncertain. It does not replace the formal deviation record.

#	What happened and when	Reason	Possible effect	Immediate response · linked record
1				
2				
3				

G Complications and device performance

Describe briefly and send on at once; the safety assessment is done in the SART.

#	Event or device concern	What was seen	Related to device or procedure?	Treatment in theatre	SART · SCML
1			☐ Device ☐ Procedure ☐ Both ☐ Not related ☐ Unknown		
2			☐ Device ☐ Procedure ☐ Both ☐ Not related ☐ Unknown		
3			☐ Device ☐ Procedure ☐ Both ☐ Not related ☐ Unknown		

Anything that may affect participant safety, device accountability or endpoint data is sent through the local and sponsor route now; it never waits for a later review.

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Post-operative review and case closure

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H Review right after surgery

Before closing the case, confirm the record is usable and every route is linked.

Item	Status	Evidence or reference	Open action · owner
Final configuration and implant position documented in the operative note and the linked image	☐ Adequate ☐ Partial ☐ Not adequate ☐ N/A		
Component list complete and reconciled with labels and the accountability record	☐ Adequate ☐ Partial ☐ Not adequate ☐ N/A		
Immediate post-operative condition documented in the source record	☐ Adequate ☐ Partial ☐ Not adequate ☐ N/A		
Post-operative image taken or ordered and logged in the IQL	☐ Adequate ☐ Partial ☐ Not adequate ☐ N/A		
Every safety, deviation and device-deficiency signal has a linked route	☐ Adequate ☐ Partial ☐ Not adequate ☐ N/A		

Summary of the review	

I Case closure

Complete: record usable and every route linked. Complete with open action: owner, due date and destination in J. Hold: not closed until the blocker is solved or escalated.

Closure status	Time	Responsible (name / role)	Linked open action or record
☐ Complete ☐ Complete with open action ☐ Hold			

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Open actions, signatures and filing

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J Open actions and follow-up

Only for an issue from B to I still open at closure. The receiving record keeps its own content; this table keeps the handoff, owner, due date and proof the action worked.

#	Issue (section)	Sent to (record)	Owner	Due date	Proof it worked	Status
1						☐ Open ☐ Closed
2						☐ Open ☐ Closed
3						☐ Open ☐ Closed
4						☐ Open ☐ Closed

K Signatures and filing

Completed by (in theatre)

Name / role:

Signature / date and time:

Operating surgeon

Name / role:

Signature / date and time:

Principal Investigator (if required)

Name / role:

Signature / date and time:

ICDL filed at	
Accountability record	
Linked safety, deviation and image records	